

Scene and Safety Management

Upon arrival	Check for Safety 1-2-3	Read the scene
- Note time of arrival - Tell control - Call for help or backup early	You - put on gloves/glasses immediately Environment - is there a crowd/other vehicles? Patient - are they being aggressive?	- Consider the Mechanism of Injury (MOI) 90% diagnosis

Primary order of first aid: ABCDE

- A** Airway (c-spine/neck)
- B** Breathing
- C** Circulation
- D** Disability
- E** Exposure and Evacuation



unless patient is screaming, then work in this order

- C** Catastrophic external bleeding
- A** Airway (c-spine/neck)
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C - Catastrophic bleeding

Tourniquet

- single loop
- double loop (thigh)
- continued bleed may need 2 tourniquets

Manual In-Line Stabilisation (MILS)

- Stabilize head and neck with:
 - knees (if possible)
 - one hand
 - nearby equipment

4 points of alignment

- Nose
- Sternum
- "fluffy bits"
- Eyes looking forward

Establish level of consciousness

- Alert**
- Verbal**
- Pain** (airway concern)
- Unresponsive to pain**

A - Airway

Is the airway noisy or silent?

Noisy = partially obstructed

- Bubbling -> check for fluid
- Snoring -> tongue blockage

- Check **hard** structures (jaw, cheek bones, teeth)
- Check **soft** tissue (lips, tongue, larynx, nose – milk for signs of bleeding)

- Open mouth and clear foreign objects
- For blood/vomit, use postural drainage then suction

Silent = complete obstruction

- back blows
- chest thrusts

Create an airway

- jaw thrust
- chin lift

If struggling, consider

- x2 NPA (6 Female / 7 Male)
- OP tube
- Keep going manually

B - Breathing

Expose chest to waistline, if possible.

Rate – Count the rate of breathing (10-30 seconds)

Volume – Assess the volume of breathing

Oxygen device

- free flow or assisted
- Ventilation depending on rate
- (CD cylinder ≈ 20 min @ 15 L/min)

Feel the chest

- Holes on the chest (cover all vent one on each side)
- Chest seal to sucking chest wound

Look at the chest

- Pattern bruising on the chest (internal problem)

Armpits

Search

- Back
- Sides
- Shoulders
- clavicles

Holes

C - Circulation

Check for location of blood and pulse.

If on the **floor**

- Stop all external bleeding
- direct pressure
- hemostatic gauze to junctional bleeds
- elevation and indirect pressure
- windlass dressing and splint long bones and pelvis

If on **chest** or **abdomen**:

- Surgeon needed asap
- If on **pelvis**:
 - Surgeon needed asap
 - Splint if unstable
 - Do NOT rock

If on **long bones**:

- femur alignment (hip, knee, 2nd toe)
- Look for signs of internal bleeding
 - pattern bruising

Check for radial **pulse**:

- Check both sides
- Check the rate
 - Capillary refill time (CRT) <2s
- Check the volume
- Check skin colour
 - pale and clammy

No pulse?

- legs to 45° (only if no femur/pelvis fracture)
- preserve the first clot by applying careful pressure

D - Disability

Reassess every 5 min

- Are they moving all 4 limbs?
- Assess disability every 5 mins

- Alert**
- Verbal**
- Pain** (airway concern)
- Unresponsive to pain**

Check **PEARL**:

Pupils Equal And Reactive to Light

- Assess analgesia need
- Pain score 1–10

- Consider Pentrox – **ABACKPAC**

- Questions: check safe with 2nd

E – Exposure/Evacuation

Keep warm, pack, wrap, evacuate

Do not overexpose patient to element, risk of hypothermia

Keep them warm

Protect the head

Prepare evacuation route